

National Coverage Determination (NCD)

PET (FDG) for Colorectal Cancer

220.6.4

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Tracking Information

Publication Number

100-3

Manual Section Number

220.6.4

Manual Section Title

PET (FDG) for Colorectal Cancer

Version Number**Effective Date of this Version**

01/28/2005

Ending Effective Date of this Version

04/03/2009

Implementation Date

04/18/2005

Description Information

Benefit Category

Diagnostic Tests (other)

Please Note: This may not be an exhaustive list of all applicable Medicare benefit categories for this item or service.

Indications and Limitations of Coverage

1. Recurrent Colorectal Carcinoma With Rising Levels of Biochemical Tumor Marker Carcinoembryonic Antigen (CEA) (Effective July 1, 1999)

Effective for services performed on or after July 1, 1999, Medicare covers FDG PET for patients with recurrent colorectal carcinomas, suggested by rising levels of the biochemical tumor marker CEA.

Frequency Limitations: Whole body PET scans for assessment of recurrence of colorectal cancer cannot be ordered more frequently than once every 12 months unless medical necessity documentation supports a separate re-elevation of CEA within this period.

Limitations: Because this service is covered only in those cases in which there has been a recurrence of colorectal tumor, claims for PET should include a statement or other evidence of previous colorectal tumor, through June 30, 2001.

2. Diagnosis, Staging, and Re-Staging (Effective July 1, 2001)

Effective for services performed on or after July 1, 2001, Medicare covers FDG PET for colorectal carcinomas for diagnosis, staging, and re-staging. New medical evidence supports the use of FDG PET as a useful tool in determining the presence of hepatic/extra-hepatic metastases in the primary staging of colorectal carcinoma, prior to selecting a treatment regimen. Use of FDG PET is also supported in evaluating recurrent colorectal cancer beyond the limited presentation of a rising CEA level where the patient presents clinical signs/symptoms of recurrence.

3. Monitoring Response to Treatment (Effective January 28, 2005)

Effective for services performed on or after January 28, 2005, Medicare only covers FDG PET for monitoring response to treatment for colorectal cancer as "coverage with evidence development".

Medicare shall notify providers and beneficiaries where these services can be accessed, as they become available, via the following:

- Federal Register Notice
- CMS coverage Web site at: www.cms.gov/coverage

Requirements: PET is covered in any/all of the following circumstances:

A. Diagnosis: PET is covered only in clinical situations in which: (1) the PET results may assist in avoiding an invasive diagnostic procedure, or in which (2) the PET results may assist in determining the optimal anatomical location to perform an invasive diagnostic procedure. In general, for most solid tumors, a tissue diagnosis is made prior to the performance of PET scanning. PET scans following a tissue diagnosis are generally performed for of staging rather than diagnosis.

B. Staging and/or Restaging: PET is covered for staging in clinical situations in which: (1)(a) the stage of the cancer remains in doubt after completion of a standard diagnostic workup, including conventional imaging (computed tomography, magnetic resonance imaging, or ultrasound), or (1)(b) the use of PET could potentially replace one or more conventional imaging studies when it is expected that conventional study information is insufficient for the clinical management of the patient, and (2) clinical management of the patient would differ depending on the stage of the cancer identified.

PET is covered for restaging after completion of treatment for the purpose of: (1) detecting residual disease, (2) detecting suspected recurrence, (3) determining the extent of a known recurrence, or (4) potentially replacing one or more conventional imaging studies when it is expected that conventional study information is insufficient for the clinical management of the patient.

C. Monitoring Response to Treatment: PET is covered for monitoring response to treatment when a change in therapy is anticipated.

Documentation that these conditions are met should be maintained by the referring physician in the beneficiary's medical record, as is normal business practice.

(This NCD last reviewed March 2005.)

Cross Reference

See NCD for [PET Scans](#) (§220.6).

Claims Processing Instructions

[TN 527 \(Medicare Claims Processing Manual\)](#)

[TN 956 \(Medicare Claims Processing\)](#)

[TN 1817 \(Medicare Claims Processing\)](#)

[TN 1833 \(Medicare Claims Processing\)](#)

Transmittal Information

Transmittal Number

31

Coverage Transmittal Link

<https://www.cms.gov/Regulations-and-Guidance/Guidance/Transmittals/Downloads/R31NCD.pdf>

Revision History

04/2005 - Covered for monitoring response to treatment for colorectal cancer as "coverage with evidence development". Effective date 01/28/05. Implementation date 04/18/05. ([TN 31](#) [↗](#)) (CR 3741)

Additional Information

Other Versions

Title	Version	Effective Between	
FDG PET for Colorectal Cancer (Replaced with Section 220.6.17) - RETIRED	3	04/10/2023 - N/A	View
FDG PET for Colorectal Cancer (Replaced with Section 220.6.17)	2	04/03/2009 - 04/10/2023	View
PET (FDG) for Colorectal Cancer	1	01/28/2005 - 04/03/2009	You are here